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- CHAPTER 27 Mental Hygiene TITLE B Mental Health Act
- ARTICLE 9 Hospitalization of Persons With a Mental Illness

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liability

Mental Illness

persons to or
between hospitals

VIEWING MOST RECENT REVISION
(FROM 2025-08-08)

SHARE



VIEW HISTORICAL REVISION AS OF:

2025-08-08





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* § 9.60 Assisted outpatient treatment.

(a) Definitions. For purposes of this section,
the following

definitions shall apply:

(1) "assisted outpatient treatment" shall
mean categories of

assertive community treatment team
services to provide care
coordination, and may also include any of
the following categories of
services: medication; periodic blood tests or
urinalysis to determine
compliance with prescribed medications;

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supervision of living arrangements; and
 any other services within a
 local services plan developed pursuant to
 article forty-one of this
 chapter, prescribed to treat the person's
 mental illness and to assist
 the person in living and functioning in the
 community, or to attempt to
 prevent a relapse or deterioration that may
 reasonably be predicted to

community services of a
 local governmental unit, or the director of
 a hospital licensed or
 operated by the office of mental health
 which operates, directs and
 supervises an assisted outpatient
 treatment program.

ARTICLE FORTY ONE OF THIS



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is a different county than the county
where the assisted outpatient
treatment order was originally issued.

(4) "assisted outpatient treatment program"
shall mean a system to
arrange for and coordinate the provision of
assisted outpatient
treatment, to monitor treatment
compliance by assisted outpatients. to

ensure compliance with court orders.

(5) "assisted outpatient" shall mean the
person under a court order to
receive assisted outpatient treatment.

(6) "subject of the petition" or "subject"

outpatient treatment.



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section two of the correction law.

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(8) "health care proxy" and "health care agent" shall have the same

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meanings as provided in article twenty-nine-C of the public health law.

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(9) "program coordinator" shall mean an individual appointed by the commissioner of mental health. pursuant

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treatment programs.

(b) Programs. The director of community services of each local governmental unit shall operate, direct and supervise an assisted outpatient treatment program. The

program, upon approval by the



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joint assisted outpatient treatment programs. Nothing in this subdivision shall be interpreted to preclude the combination or coordination of efforts between and among local governmental units and hospitals in providing and coordinating assisted outpatient treatment.

(c) Criteria. A person may be ordered to

(1) is eighteen years of age or older; and

(2) is suffering from a mental illness; and

(3) is unlikely to survive safely in the community without supervision, based on a clinical

THAT HAS.



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petition, at least twice within the last thirty-six months been a significant factor in necessitating hospitalization in a hospital, or receipt of services in a forensic or other mental health unit of a correctional facility or a local correctional facility, not including any current period, or period ending within the last six months. during

subparagraph (iii) of this paragraph, prior to the filing of the petition, resulted in one or more acts of serious violent behavior toward self or others or threats of, or attempts at, serious physical harm to self or others within the last

of incarcerated, or



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assisted outpatient

treatment that has expired within the last six months, and since the expiration of the order; (a) the person has experienced a substantial increase in symptoms of mental illness that substantially interferes with or limits the person's ability to comply with recommended treatment: or (b) the person. due to a lack

inpatient care or has been incarcerated;

(5) is, as a result of his or her mental illness, unlikely to voluntarily participate in outpatient treatment that would enable him or

in need of assisted outpatient treatment in



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of this article; and

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(7) is likely to benefit from assisted outpatient treatment. Previous

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non-compliance with court oversight or mandated treatment shall not

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preclude a finding that the person is likely to benefit from assisted outpatient treatment.



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this chapter and consistent with article twenty-nine-C of the public health law.

(e) Petition to the court. (1) A petition for an order authorizing assisted outpatient treatment may be filed

petition may be initiated only



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the petition resides; or

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(ii) the parent, spouse, domestic partner,
sibling eighteen years of

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age or older, or child eighteen years of age
or older of the subject of

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the petition; or

(iii) the director of a hospital in which the
subject of the petition

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home providing mental health services to
the subject of the petition or
in whose institution the subject of the
petition resides; or

(v) a qualified psychiatrist who is either
supervising the treatment

ARTICLE ONE PART TWO



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the subject of the petition for a mental
illness; or

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(vii) the director of community services, or
his or her designee, or

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the social services official, as defined in the
social services law, of

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the city or county in which the subject of
the petition is present or
reasonably believed to be present: or

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(2) The petition shall state:

(i) each of the criteria for assisted
outpatient treatment as set
forth in subdivision (c) of this section;

petition need not be limited to the stated



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believed to be present, within the county
where such petition is filed.

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(3) The petition shall be accompanied by an
affirmation or affidavit
of a physician, who shall not be the
petitioner, stating either that:

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(i) such physician has personally examined
the subject of the petition

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petition, and is willing and able to testify at
the hearing on the
petition; or

(ii) no more than ten days prior to the filing
of the petition, such
physician or his or her designee has made

physician has reason to



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the subject of the petition and testify at
the hearing on the petition.

(4) In counties with a population of less
than eighty thousand, the
affirmation or affidavit required by
paragraph three of this subdivision
may be made by a physician who is an
employee of the office. The office
is authorized to make available. at no cost

paragraph.

(f) Service. The petitioner shall cause
written notice of the petition
to be given to the subject of the petition
and a copy thereof to be
given personally or by mail to the persons

the appropriate program



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(g) Right to counsel. The subject of the petition shall have the right to be represented by the mental hygiene legal service, or privately financed counsel, at all stages of a proceeding commenced under this section.

(h) Hearing. (1) Upon receipt of the petition, the court shall fix the

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Sundays and holidays. Adjournments shall be permitted only for good cause shown. In granting adjournments, the court shall consider the need for further examination by a physician or the potential need to provide assisted outpatient treatment

petitioner, the physician whose



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date, or upon such other date to which the proceeding may be adjourned, the court shall hear testimony and, if it be deemed advisable and the subject of the petition is available, examine the subject of the petition in or out of court. If the subject of the petition does not appear at the hearing, and appropriate attempts to elicit the attendance

basis for conducting the hearing without the presence of the subject of the petition.

(2) The court shall not order assisted outpatient treatment unless an examining physician, who recommends

LEGISLATION BY PERSON OR BY



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that diligent efforts have been made to attend such hearing in person and the subject of the petition consents to the physician testifying by video conference; or (ii) the court orders the physician to testify by video conference upon a finding of good cause. Such physician shall state the facts and clinical determinations which support the allegation

(3) If the subject of the petition has refused to be examined by a physician, the court may request the subject to consent to an examination by a physician appointed by the court. If the subject of the petition does not consent and the court

then special duties, or police



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custody and transport him or her to a hospital for examination by a physician. Retention of the subject of the petition under such order shall not exceed twenty-four hours. The examination of the subject of the petition may be performed by the physician whose affirmation or affidavit accompanied the petition pursuant to paragraph three of

examination is performed by another physician, the examining physician may consult with the physician whose affirmation or affidavit accompanied the petition as to whether the subject meets the criteria for assisted outpatient treatment.

support the allegation that



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(iii) the recommended assisted outpatient treatment, and (iv) the rationale for the recommended assisted outpatient treatment. If the recommended assisted outpatient treatment includes medication, such physician's testimony shall describe the types or classes of medication which should be authorized, shall describe the beneficial and

administered by authorized personnel.

(5) The subject of the petition shall be afforded an opportunity to present evidence, to call witnesses on his or her behalf, and to cross-examine adverse witnesses.

appointed by the appropriate



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shall include case management services or assertive community treatment team services to provide care coordination. The written treatment plan also shall include all categories of services, as set forth in paragraph one of subdivision (a) of this section, which such physician recommends that the subject of the petition receive. All service providers shall be

medication should be self-administered or administered by authorized personnel, and shall specify type and dosage range of medication most likely to provide maximum benefit for the subject. If the written treatment plan includes alcohol or

provided the physician



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substance abuse that is clinically related to the mental illness; and

(ii) that such testing is necessary to prevent a relapse or deterioration which would be likely to result in serious harm to the person or others. If a director is the petitioner, the written treatment plan shall be provided to the court no later than the date of the

date set by the court pursuant to paragraph three of subdivision (j) of this section.

(2) The physician appointed to develop the written treatment plan shall provide the following persons with an

upon the request of the



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with the welfare of the subject. If the subject of the petition has executed a health care proxy, the appointed physician shall consider any directions included in such proxy in developing the written treatment plan.

(3) The court shall not order assisted outpatient treatment unless a

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categories of assisted outpatient treatment recommended, the rationale for each such category, facts which establish that such treatment is the least restrictive alternative, and, if the recommended assisted outpatient treatment plan includes

SUCH MEDICATION, AND WHETHER



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health care proxy, such physician shall state the consideration given to any directions included in such proxy in developing the written treatment plan. If a director is the petitioner, testimony pursuant to this paragraph shall be given at the hearing on the petition. If a person other than a director is the petitioner, such testimony shall be

(j) Disposition. (1) If after hearing all relevant evidence, the court does not find by clear and convincing evidence that the subject of the petition meets the criteria for assisted outpatient treatment, the court shall dismiss the petition.

OF THE PETITION MEETS THE



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subject to receive assisted outpatient treatment for an initial period not to exceed one year. In fashioning the order, the court shall specifically make findings by clear and convincing evidence that the proposed treatment is the least restrictive treatment appropriate and feasible for the subject. The order shall state an assisted outpatient

of this section, which the assisted outpatient is to receive, but shall not include any such category that has not been recommended in both the proposed written treatment plan and the testimony provided to the court pursuant to subdivision (i) of this section.

clear and convincing evidence



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written proposed treatment plan and testimony pursuant to subdivision (i) of this section, the court shall order the appropriate director to provide the court with such plan and testimony no later than the third day, excluding Saturdays, Sundays and holidays, immediately following the date of such order. Upon receiving such plan and testimony, the

(4) A court may order the patient to self-administer psychotropic drugs or accept the administration of such drugs by authorized personnel as part of an assisted outpatient treatment program. Such order may specify the type and dosage range of such



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hospital director to provide or arrange for all categories of assisted outpatient treatment for the assisted outpatient throughout the period of the order. In all other instances, the order shall require the appropriate director, as that term is defined in this section, to provide or arrange for all categories of assisted outpatient treatment

court order issued pursuant to this section to be served personally, or by mail, facsimile or electronic means, upon the assisted outpatient, the mental hygiene legal service or anyone acting on the assisted outpatient's behalf, the



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director shall review whether the assisted outpatient continues to meet the criteria for assisted outpatient treatment. If, as documented in the petition, the director determines that such criteria continue to be met or has made appropriate attempts to, but has not been successful in eliciting, the cooperation of the subject to submit to an examination.

continued assisted outpatient treatment pursuant to paragraph two of this subdivision. Upon determining whether such criteria continue to be met, such director shall notify the program coordinator in writing as to whether a petition for continued assisted

(2) within thirty days prior to the



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pursuant to subparagraph
(i) or (ii) of paragraph one of subdivision (e)
of this section, and the
current petitioner retains his or her
original status pursuant to the
applicable subparagraph, may petition the
court to order continued
assisted outpatient treatment for a period
not to exceed one year from
the expiration date of the current order. If

effect until such disposition.

The procedures for obtaining any order
pursuant to this subdivision
shall be in accordance with the provisions
of the foregoing subdivisions
of this section; provided that the time
restrictions included in

SUBDIVISION (j) OF THIS SECTION SHALL BE



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after six months by the physician who developed the written treatment plan or another physician designated by the director, and such physician shall be authorized to terminate such blood tests or urinalysis without further action by the court.

(l) Petition for an order to stav. vacate or

assisted outpatient, the mental hygiene legal service, or anyone acting on the assisted outpatient's behalf may petition the court on notice to the director, the original petitioner, and all others entitled to notice under subdivision (f) of

the court for approval



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court. Such petition shall be filed on notice to all parties entitled to notice under subdivision (f) of this section. Not later than five days after receiving such petition, excluding Saturdays, Sundays and holidays, the court shall hold a hearing on the petition; provided that if the assisted outpatient informs the court that he or she agrees to

without court approval. For the purposes of this paragraph, a material change is an addition or deletion of a category of services to or from a current assisted outpatient treatment plan, or any deviation without the assisted outpatient's consent from the

(iii) APPEALS. REVIEW OF AN ORDER ISSUED



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(n) Failure to comply with assisted outpatient treatment. Where in the clinical judgment of a physician, (i) the assisted outpatient, has failed or refused to comply with the assisted outpatient treatment, (ii) efforts were made to solicit compliance, and (iii) such assisted outpatient may be in need of involuntary admission to a hospital

physician may request the appropriate director of community services, the director's designee, or any physician designated by the director of community services pursuant to section 9.37 of this article, to direct the removal of such assisted outpatient to

pursuant to section 9.41, 9.39 or



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to take, or fails a blood test, urinalysis, or alcohol or drug test as required by the court order, such physician may consider such refusal or failure when determining whether the assisted outpatient is in need of an examination to determine whether he or she has a mental illness for which hospitalization is necessary. Upon the request of such physician.

officers, acting pursuant to their special duties, or police officers who are members of an authorized police department or force or of a sheriff's department to take the assisted outpatient into custody and transport him or her to the hospital

persons. Such law enforcement



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physician designated pursuant to section

9.37 of this article, an

ambulance service, as defined by

subdivision two of section three

thousand one of the public health law, or

an approved mobile crisis

outreach team as defined in section 9.58 of

this article shall be

authorized to take into custody and

transport any such person to the

services to receive such persons. Any

director of community services, or

designee, shall be authorized to direct the

removal of an assisted

outpatient who is present in his or her

county to an appropriate

hospital, in accordance with the provisions

outpatient pursuant to this



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hours to permit a physician to determine whether such person has a mental illness and is in need of involuntary care and treatment in a hospital pursuant to the provisions of this article. Any continued involuntary retention in such hospital beyond the initial seventy-two hour period shall be in accordance with the provisions of this article

to meet the involuntary admission and retention provisions of this article, and does not agree to stay in the hospital as a voluntary or informal patient, he or she must be released. Failure to comply with an order of assisted outpatient treatment

(c) Effect of determination that a person is



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not be construed as or
deemed to be a determination that such
person is incapacitated pursuant
to article eighty-one of this chapter.

(p) False petition. A person making a false
statement or providing
false information or false testimony in a
petition or hearing under this
section shall be subject to criminal

(q) Exception. Nothing in this section shall
be construed to affect
the ability of the director of a hospital to
receive, admit, or retain
patients who otherwise meet the
provisions of this article regarding
receipt, retention or admission.

administration, shall prepare



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services, judges, court personnel, law enforcement officials and the general public.

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(2) The office, in consultation with the office of court

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administration, shall establish a mental health training program for supreme and county court judges and court personnel. Such training shall

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(s) A director of community services or his or her designee may require a provider of inpatient psychiatric services operated or licensed by the office of mental health to provide contemporaneous

assisted outpatient treatment pursuant to



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the care of any individual who
was subject to an active assisted outpatient
treatment order to appro-
priately discharge their duties pursuant to
section 9.47 of this
article, and where such provider of
inpatient psychiatric services is
required to disclose such information
pursuant to paragraph twelve of
subdivision (c) of section 33.13 of this

information obtained by the
director of community services pursuant
to this subdivision shall be
public records, and the records shall not be
released by the director to
any person or agency, except as already
authorized by law.

* NB Repealed June 30, 2027



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